

“For far too many, **pregnancy and birth** is still something that **happens to them** rather than something they set out **consciously** and **joyfully to do** themselves.”

SHEILA KITZINGER, *THE EXPERIENCE OF CHILDBIRTH*, 1962

approaches to pain relief. Arabic women learned Raqs Sharqi (known in the west as belly dancing) from puberty. This dance had a threefold purpose: it increased abdominal strength from a young age; the movements reduced pain by helping the baby move more smoothly into the birth canal; and other women would also join the dance, attempting to hypnotize the mother into following their movements and keeping time. Women in West Micronesia began dilation artificially around a month before the mother's due date, using tightly rolled leaves. This decreased the pain, and shortened labor considerably.

Pushing through

Throughout history, one of the greatest concerns about childbirth has been the survival of both mother and child. Many factors also influenced a baby's chance of survival. Infants born to poor women and slaves had a higher chance of fatality because of maternal malnutrition and disease. In the 18th century, the mortality rate for children born to African American slaves was as high as 50 percent, although some sources suggest that women smothered their babies to spare them a life in bondage. In some societies, infant and maternal mortality have been seen as punishments. On the Yucatán Peninsula, husbands are still expected to witness the suffering of the woman during labor and, if absent, are blamed for deaths.

Advances in medical equipment have contributed to a decrease in infant death rates. A common cause of infant mortality was becoming stuck in the descent into the birth canal. In the 16th century, French obstetrician Peter Chamberlen developed forceps to help

extract the baby from the birth canal, thereby preventing suffocation. His family kept the invention a secret until the 17th century, when its use became commonplace.

Other birth procedures have a much longer history. Evidence suggests, for example, that the caesarean section took place as many as 3,000 years ago in China and 2,200 years ago in India. At first the procedure was practiced to save the infant if the mother had died during labor because, before 1500, surgical techniques and hygiene practices were not sophisticated enough to allow the mother to survive the operation. Today, increased knowledge of hygiene and anatomy has made childbirth safer than ever before, but infant mortality rates remain high in less developed parts of the world.

“[My husband] **cut a hole in the wooden bed** and placed a caribou skin over it for me to **release the baby**.”

APPHIA AWA, INUIT WOMAN FROM NUNAVUT, CANADA, 1992



▲ **BIRTHING CHAIR**
Many women in history gave birth sitting down with the aid of furniture such as this 17th-century French chair. It could also be folded out flat to become a bed or an operating table.